National Cancer Control Programme (NCCP) 00798 · clinician draft

Phesgo + vinorelbine

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

intravenous (IV) vinorelbine · Days 1 and 8

Active medicines: Pertuzumab / trastuzumab (Phesgo) + IV vinorelbine (HER2 treatment on Day 1 only, if still prescribed)

Emetogenic risk: Minimal · IV route

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- If HER2 therapy has stopped, the IV or oral vinorelbine plan may still apply, but remove the stopped antibodies from the prescription. Confirm the active components at every visit.
- Confirm Days 1 and 8 only, active vinorelbine route, current counts and the actual prescribed dose. IV and oral doses are not interchangeable.
- Assess constipation; agree prophylactic laxative or dietary support where appropriate. Severe pain, vomiting or no stool/wind requires urgent review.
- Antibody premedication is usually unnecessary unless indicated by previous hypersensitivity; consider paracetamol/antihistamine per local policy.
- No routine IV taxane steroid or carboplatin antiemetic course applies. An individual rescue plan may still be needed.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00798 v3 · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-21 · SACTCheck v0.84.16 · Regimen-level drafts; prescriber and oncology-pharmacy verification required